



PROCEDURE

Transfer and Care of Sedated Patients to Ward 5A

Scope (Staff):	Medical, Nursing
Scope (Area):	Perth Children's Hospital (PCH) - Emergency Department (ED), Paediatric Critical Care Unit (PCC) and Ward 5A

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Only use this box if relevant (e.g. clinical document or available on an external website)

Aim

To ensure that deeply sedated children transferring from an external facility to CAMHS Inpatient Service Ward 5A for mental health assessments, receive a medical clearance before admission to Ward 5A; and to ensure that appropriate assessments are continued while on Ward 5A.

Please note that sedated children will not be accepted for admission to CAMHS Inpatient Service on Nickoll Ward.

Risk

Inadequate assessment of the deeply sedated patient's clinical condition may result in adverse patient outcomes related to over sedation and respiratory compromise.

Background

On occasion, sedation is required to manage patients with acute behavioural disturbance. The level of sedation required may range from orally administered conscious sedation/anxiolysis to parenterally administered deeper sedation. The progression from conscious sedation and deep sedation through to anaesthesia may be rapid and unpredictable. Inadvertent deep sedation carries significant risks and requires the necessary skills to be able to manage unconscious patients including monitoring of relevant physiological variables.¹ The scope of this guideline is applicable to children who have undergone deep sedation for the purpose of interfacility transfer.

Definitions

Conscious Sedation: a drug-induced depression of consciousness during which patients are able to respond purposefully to verbal commands or light tactile stimulation. Interventions to maintain a patent airway, spontaneous ventilation or cardiovascular function may, in exceptional situations, be required.²

Deep Sedation: is characterised by depression of consciousness that can readily progress to the point where consciousness is lost and patients respond only to painful stimulation. It is associated with loss of the ability to maintain a patent airway, inadequate spontaneous ventilation and/or impaired cardiovascular function, and has similar risks to general anaesthesia, requiring an equivalent level of care.²

Medical Clearance: where the patient has been assessed by the referring/treating team and deemed 'safe' for any necessary activity or transfer. See [Medical Clearance Prior to Ward 5A Transfer](#) for further detail.

Sedation: For the purpose of this procedure, sedation refers to medication given for the purpose of effecting somnolence, anxiolysis or motion control irrespective of dose and route of administration.

University of Michigan Sedation Scale (UMSS): an observation tool that assesses the level of alertness using a five-point scale ranging from 1 (fully alert) to 5 (unrousable with deep stimulation).³

Key points

- All patients transferring from an external facility to Ward 5A for assessment (including Royal Flying Doctor Service (RFDS transfers) who are under the age of 16 years and who have undergone deep sedation must present to the PCH Emergency Department (ED) or the Paediatric Critical Care Unit (PCC) for medical clearance prior to acceptance on Ward 5A.
- This requirement also applies to patients who are transferred on Mental Health Act forms, even though Ward 5A is identified as the place of referral on the form.

Medical Clearance Prior to Ward 5A Transfer

- Intubated and ventilated patients must be admitted to the Paediatric Critical Care (PCC) Unit for assessment and extubation.
- Non-intubated patients will be assessed in the Emergency Department (ED).
- Assessment for medical clearance should include;
 - Vital signs within acceptable parameters on the relevant PARROT chart;
 - No Code Blue in the preceding 24 hours;
 - Consideration of communicable diseases / infection control needs;
 - No acute treatments, monitoring or nursing care required for the duration of the activity; and

- Vital signs and other parameters of patients with known disordered eating in the Green range at the time of referral as per the pre-referral guidelines for Eating Disorders Service (EDS).

Acceptance Criteria for Transfer to Ward 5A

Ward 5A is not a medical unit; the management of medical devices remains outside the scope of Ward 5A nursing staff and relevant equipment and consumables are not available in the unit. Therefore the following are the minimum requirements that must be met prior to transfer for admission on Ward 5A:

- Medically cleared by PCH ED or Ward 3A (PCC).
- No oxygen dependency including use of nasal prongs.
- Endotracheal tubes must be removed 4 hours prior to transfer, unless earlier discharge is approved by the PCC Consultant and has been discussed with the Ward 5A Consultant Psychiatrist.
- No medical devices in situ, e.g. intravenous cannula.
- UMSS score ≤ 2 .
- Indwelling urinary catheter removed and the patient able to void comfortably.

Monitoring on Ward 5A

- On admission to Ward 5A, an assessment using the Observation, Monitoring and [Paediatric Acute Recognition and Response Observation Tool \(PARROT\)](#) must be undertaken in accordance with the PCH Procedure.
- At a minimum, hourly monitoring of respiratory rate, pulse, temperature, blood pressure and oxygen saturation should be undertaken for the first 6 hours; and then four hourly for the next 18 hours. Refer to the PCH [Procedural Sedation Guideline](#) for further information regarding post sedation monitoring requirements.
- The patient must be assessed by the Ward 5A Registrar or (or PCH on-call Registrar) within 4 hours of transfer and regularly thereafter as clinically indicated. Refer to [Admission to CAMHS Ward 5A Procedure](#).

Escalation in Case of Deterioration

Concerns regarding the patient's clinical condition are to be escalated in accordance with the [Observation, Monitoring and Paediatric Acute Recognition and Response Observation Tool \(PARROT\) procedure](#).

Refer also to the following PCH policy documents:

- [STARS Guideline](#)
- [Resuscitation and Responding to Clinical Deterioration MET Review and Code Blue](#)

Related CAHS internal policies, procedures and guidelines
<u>Admission to CAMHS Ward 5A Procedure</u>
<u>Arousal and Agitation Drug Management Guideline</u>
<u>Clinical Handover Procedure – PCH Multidisciplinary</u>
<u>Intrahospital Transfer Guideline</u>
<u>Procedural Sedation Guideline</u>
<u>Paediatric Acute Recognition and Response Observation Tool (PARROT)</u>
<u>Patient / Client Identification Protocol</u>
<u>Physical Health Assessment and Monitoring Policy</u>
<u>Pre-referral guidelines for CAMHS Eating Disorders Service</u>
<u>Resuscitation and Responding to Clinical Deterioration MET Review and Code Blue</u>
<u>STARS Guideline</u>

Related external legislation, policies, and guidelines
<u>Medicines and Poisons Act 2014</u>
<u>Mental Health Act 2014 (WA)</u>
<u>Clinical Incident Management Policy</u> - MP 0122/19
<u>Consent to Treatment Policy</u> - MP 0175/22
<u>High Risk Medication Policy</u> - MP 0131/20
<u>Medication Chart Policy</u> - MP 0078/18
<u>Medication Review Policy</u> - MP 0104/19
<u>Recognising and Responding to Acute Deterioration (RRAD) Policy</u> - MP 0171/22
<u>Road-Based Transportation for Mental Health Consumers Policy</u> - MP 0063/17
<u>Safety Planning for Mental Health Consumers</u> – MP 018/24
<u>Use of Physical and/or Mechanical Restraint during Road-based Transportation of Mental Health Patients Policy</u> - MP 0165/21

References and related external legislation, policies, and guidelines

Australian New Zealand College of Anaesthetics (ANSCA). Guideline for Safe Care of Patients Sedated in Health Care Facilities for Acute Behavioural Disturbance. 2019.

Australian New Zealand College of Anaesthetics (ANSCA). Guidelines for Sedation and/or Analgesia for Diagnostic and Interventional Medical, Dental or Surgical Procedures. 2014.

Malviya S, Voepel-Lewis T, Tait AR, Merkel S, Tremper K, Naughton N. Depth of sedation in children undergoing computed tomography: validity and reliability of the University of Michigan Sedation Scale UMSS. British Journal of Anaesthesia, 88 (2), 241-245, 2002.

This document can be made available in alternative formats on request.

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